INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Atezolizumab + bevacizumab for hepatocellular carcinoma (HCC)

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00831
Version 2a

Treatment every three weeks

Treatment stages

01 **Atezolizumab then bevacizumab**
Day 1 every three weeks

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	11
04	Your record and contact card	18
05	Sources and further information	22

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your oncology team immediately for new breathlessness, persistent diarrhoea, blood or black stools, yellow eyes, dark urine, severe tiredness or dizziness, thirst and frequent urination, unusual weakness, reduced urine or a spreading rash. Tell every healthcare professional that you receive immunotherapy, even after it stops. Vomiting blood, heavy bleeding, sudden severe abdominal or chest pain, stroke symptoms, seizure, collapse or major breathing difficulty need 112 / 999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

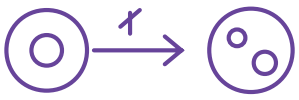
Two medicines with different effects and safety checks

This combination aims to control hepatocellular carcinoma, a cancer that starts in the liver, when it is advanced or cannot be removed and no previous systemic treatment has been given. Atezolizumab helps immune cells recognise cancer; bevacizumab blocks a signal that supports blood-vessel growth.

PD-L1 IMMUNE CHECKPOINT INHIBITOR

Atezolizumab

Tecentriq; intravenous (IV) infusion or thigh injection



Helps immune cells recognise and attack cancer

Blocking PD-L1 can release a brake on the immune response. This may control cancer but can also cause inflammation in healthy organs during treatment or after it stops.

PD-L1 blockade → Immune response increases → Cancer and healthy tissues may be affected

What to watch for

Carry the immunotherapy alert card. Report new bowel, breathing, skin, hormone, heart, liver, kidney or neurological symptoms promptly.

ANTI-VEGF ANTIBODY

Bevacizumab

Avastin or prescribed biosimilar; IV infusion



Blocks a signal for blood-vessel growth

Bevacizumab binds VEGF, reducing signals that help tumours develop blood vessels. It can also affect normal blood vessels, blood pressure, kidney filtration and wound healing.

VEGF signal blocked → Tumour vessel growth reduced → Blood vessels and healing need monitoring

What to watch for

Report bleeding, severe headache, chest or abdominal pain, leg swelling, reduced urine or poor wound healing. Tell the team before surgery or dental procedures.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding the combination

Confirm the treatment aim and source eligibility.

Liver health matters

The team checks liver function, cirrhosis, viral-hepatitis management and any ascites or confusion. The pivotal study mainly supports patients with Child-Pugh A liver function; poorer liver function needs specialist discussion.

Two medicine decisions

There is no fixed total number of cycles. Clinical benefit, liver function, bleeding risk, immune effects and your preferences guide treatment. One medicine may be paused or stopped while the other is considered separately by the team.

Evidence in previously untreated unresectable HCC

IMbrave150 compared atezolizumab plus bevacizumab with sorafenib in selected people with HCC and preserved liver function.

The combination improved survival and time before cancer progressed in the study population. This does not predict an individual outcome or establish the same benefit for more severe liver dysfunction.

CHAPTER 02

Your schedule, tests and appointments

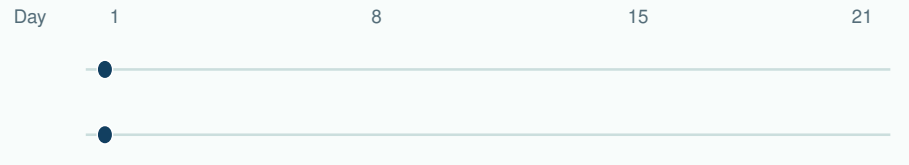
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment, monitoring and review every three weeks

Atezolizumab + bevacizumab for HCC

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAY 1 EVERY THREE WEEKS

Atezolizumab then bevacizumab

D1 | D2–20 | D21

Atezolizumab is given first on Day 1 as an IV infusion or a subcutaneous injection into the thigh, followed by bevacizumab through a vein. Treatment is planned every 21 days while it is helping and side effects remain acceptable.

BASELINE → TREATMENT → FOLLOW-UP

Checks through your treatment

Baseline preparation, checks before each cycle and tests arranged because of symptoms are different. No fixed scan interval is imposed here.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00831 v2a · official regimen

Tests in your plan

Alpha-fetoprotein (AFP)

If part of your plan

WHEN

If it was raised and is informative for your tumour; your team sets the dates.

A selected marker interpreted with imaging and the clinical review.

Other liver conditions can raise AFP, and a normal result does not exclude cancer.

THE WHOLE TREATMENT JOURNEY

Atezolizumab + bevacizumab for HCC: treatment and checks

First-line advanced or unresectable liver cancer (HCC)

Atezolizumab is given first on Day 1 as an IV infusion or a subcutaneous injection into the thigh, followed by bevacizumab through a vein. Treatment is planned every 21 days while it is helping and side effects remain acceptable.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Each 21-day cycle	Between visits / six-week checks	Benefit and liver review	After final treatment
Treatment course NCCP 00831	Confirm first-line HCC and route	Atezolizumab IV/ subcutaneous (SC) first, then IV bevacizumab	Review toxicities; no take-home anticancer tablets	Assess each component separately	Agree follow-up / next treatment
Blood count, kidney, liver and glucose NCCP 00831	Baseline tests	Repeat before every cycle	Earlier tests for symptoms or abnormalities	Distinguish underlying disease from treatment effects	Follow unresolved abnormalities
Varices, pressure and urine NCCP 00831	Varices screening/ management; blood pressure (BP) and dipstick	Dipstick before each cycle; BP before and after treatment	24-hour urine if indicated; report bleeding/ headache	Review bleeding, BP, kidney and procedure risks	Monitor until risk/ abnormalities resolve
Thyroid / immune symptoms NCCP 00831	Baseline thyroid tests and symptom review	Review symptoms and steroid/ replacement medicines	thyroid function tests (TFTs) every six weeks; extra endocrine tests if indicated	Confirm recovery and any restart criteria	Immune effects may start or continue after stopping
Cardiac / anticoagulation NCCP 00831	Cardiac history; consider echocardiogram (heart ultrasound) (ECHO) after chest radiation/ anthracycline	international normalised ratio (blood-clotting test) (INR) if indicated; warfarin plan	Weekly INR until warfarin dose stable, then each cycle	Investigate new cardiac symptoms urgently	Follow lasting effects

What each check is for

1



Varices review before bevacizumab

Before first dose

Liver disease and anti-VEGF therapy can combine to increase bleeding risk.
Ask / **act:** Confirm the screening result and management plan.

2



Review each medicine

Before every cycle

Immune and anti-VEGF toxicities have different actions.
Ask / **act:** Record which medicine is held, continued or stopped.

3 **Six-week thyroid monitoring**

During treatment

Thyroid dysfunction can develop without obvious early symptoms.
Ask / **act:** Confirm who books and reviews results.

4 **After treatment**

After last dose

Immune effects may be delayed and other risks persist.
Ask / **act:** Agree contact, contraception and medicine follow-up.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Before travelling

Confirm blood/urine test arrangements, blood-pressure checks, transport and the 24-hour number. Tell the team if you are unwell before attending.

STEP 2

Time at the unit

IV atezolizumab is usually infused over 60 minutes initially and may take 30 minutes later if tolerated. The alternative 1875 mg thigh injection takes about seven minutes. Bevacizumab is still an IV infusion: usually 90 minutes first, then 60 and later 30 minutes if tolerated. Checks, preparation and observation make the whole visit longer.

STEP 3

At home

Keep a medicines list, symptom notes and the immune alert card. Follow your own hospital contact plan; the pocket card complements the medicine alert card.

STEP 4

Procedures and collections

Tell the team about planned surgery or dental procedures and ask for clear timed urine-collection instructions if needed.

Preparing for the first treatment

Bleeding-risk review is a separate safety step.

Varices

Screening and any required variceal treatment must be reviewed before starting. Call urgently for black stools or blood; vomiting blood or heavy bleeding needs emergency help.

Baseline checks

Blood count, kidneys, liver, glucose, thyroid function, urine protein, blood pressure and cardiac history are reviewed. Other tests depend on symptoms, earlier treatment and medicines.

Consent and route

Discuss expected benefit and uncertainty, alternatives including not proceeding, liver disease and variceal bleeding, immune toxicity that may be delayed or need lifelong hormone replacement, vascular/kidney/bowel/healing risks, fertility and pregnancy prevention, IV versus thigh administration and your right to reconsider.

Follow-up after treatment

Continue the contact and alert-card plan.

Delayed immune effects

No. They can begin or persist afterwards. Keep the immunotherapy alert card and report new symptoms, including to doctors outside oncology.

Continuing support

The team follows ongoing liver, kidney, blood-pressure and hormonal problems and agrees medicine stop dates and next cancer reviews.

Pregnancy prevention

Discuss fertility and pregnancy prevention. Atezolizumab requires effective contraception for five months after the last dose and bevacizumab for six months; use the longer interval when both apply. Do not breastfeed during treatment or for at least six months after bevacizumab.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Atezolizumab	 **Day 1** IV or SC first, after review
Bevacizumab	 **Day 1** IV after atezolizumab
Symptoms and support	 **Days 1–21** Report changes at any time
Next-cycle review	 **Days 20–21** Tests and review before the next authorised cycle; appointments vary

Before first treatment: Complete variceal screening and management, baseline blood/urine tests and blood-pressure review.

After stopping: Immune effects may be delayed; keep the alert card and contact plan.

During each cycle: Symptoms can occur at any time; thyroid monitoring every six weeks is distinct from every-cycle blood tests.

> **Why these bands stay broad:** This is a reference schedule. It does not identify safe days or predict when side effects will occur. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.
> NCCP 00831 v2a · official regimen ↗

> **There are no guaranteed "safe days".** Use your own oncology unit's 24-hour contact plan. Urgent symptoms override the diary; do not wait for the next appointment.

Between-treatment notes

Day 1: Treatment day

IV atezolizumab is usually infused over 60 minutes initially and may take 30 minutes later if tolerated. The alternative 1875 mg thigh injection takes about seven minutes. Bevacizumab is still an IV infusion: usually 90 minutes first, then 60 and later 30 minutes if tolerated. Checks, preparation and observation make the whole visit longer. Report any reaction immediately.

Day 2–20: Between visits

Continue prescribed support, note symptoms and carry the immune alert card. Immune and blood-vessel effects can happen between appointments. Use the urgent contact plan; do not wait for the next cycle.

Day 21: Review before another cycle

Review symptoms, blood and urine results, blood pressure, liver health and benefit. Thyroid tests are scheduled every six weeks. Confirm each medicine and any changes.

Support around the medicines

1 

Blood pressure and urine

Blood pressure and urine-protein checks are part of each cycle. Continue prescribed blood-pressure medicines and ask before changing them. A positive urine dipstick may need a timed urine collection.

2

Immune alert card

Carry the supplied immunotherapy alert card during treatment and afterwards. New symptoms can need urgent treatment; do not wait for routine blood tests.

3 

Support medicines

Both medicines have minimal sickness risk and routine preventive anti-sickness medicines or premedication are not usually required. The team individualises symptom treatment; do not self-treat new significant diarrhoea without advice.

4

Surgery, dental work and healing

Tell oncology before planned procedures and tell your dentist about bevacizumab. Wounds must heal and the team must agree an interruption/restart plan.

Your treatment visit

The medicines follow a defined sequence.

During administration

Tell the nurse immediately about rash, swelling, breathlessness, chest tightness, dizziness or pain at the injection or infusion site. Do not wait until the visit ends.

Before every cycle

Blood count, kidney/liver profile, glucose and urine protein are checked before every cycle. Blood pressure is checked before each cycle and after treatment. Thyroid tests are scheduled every six weeks.

Care between visits

Symptoms can need review between blood tests.



Blood pressure and urine



Support medicines

Questions worth asking

Does having more side effects mean treatment is working?

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be treated. Your team assesses benefit using the tests and review plan appropriate to your cancer.

Why might a dose be delayed or reduced?

Your team may allow recovery, investigate a symptom or reduce the chance of lasting harm. Ask which drug changes, why, and when review is planned. A delay does not itself mean treatment has failed.

What should I say when calling?

Give the regimen name, last treatment date, symptoms and when they started. Explain whether you can eat, drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect details.

What belongs in the consent conversation?

The individual aim and likely benefit, uncertainty, reasonable alternatives including not proceeding, common and serious risks, lasting effects, fertility and practical concerns. You can ask for an interpreter, more time or another explanation and can reconsider your decision.

How many cycles should I use this passport for?

The passport provides reusable recording pages, not a prescribed course length. Your oncology team sets the duration and can provide extra pages.

Can I use this page to decide if blood results are safe?

No. Treatment decisions require the clinical protocol, symptoms, previous toxicity, organ function and the prescriber’s review. The patient page explains the checks and does not authorise treatment.

Why do I need a check for varices?

Liver disease can cause enlarged veins in the food pipe or stomach that may bleed. Bevacizumab can increase bleeding risk. The team checks for varices and treats any relevant risk before starting; having the test alone does not mean the risk has been controlled.

Does an abnormal liver test always mean immune hepatitis?

No. Liver cancer, underlying liver disease, infection, blocked bile flow and medicines can all affect results. The team compares results with your baseline and investigates the cause before deciding how each medicine changes.

Can one medicine be held while the other continues?

Sometimes. The reason for holding treatment, your general condition and the source rules for each medicine determine the plan. Never assume that stopping one means the other is automatically safe to continue.

Does the thigh injection replace every drip?

No. Atezolizumab may be given into the thigh, but bevacizumab remains an infusion into a vein. The two atezolizumab formulations have different doses and must be prescribed correctly.

> **Try explaining the plan back:** Can you explain which medicine is immunotherapy, why varices, blood pressure and urine are checked, which symptoms need urgent contact, and why the alert card remains important after treatment stops?

Atezolizumab

- Immune inflammation of lungs, bowel, liver, kidneys or skin
- Hormone changes, including thyroid, adrenal or pituitary problems and diabetes; replacement may be long term
- Rare serious heart, muscle, nerve or brain inflammation
- Infusion/injection reactions; delayed immune effects after stopping

Bevacizumab

- Raised blood pressure; rarely hypertensive crisis or PRES
- Protein in urine and kidney injury, including nephrotic syndrome
- Bleeding, including serious bleeding from varices
- Arterial or venous blood clots
- Poor wound healing, bowel perforation or fistula; rare jaw problems
- Infusion or allergic reactions

Treatment scope · NCCP 00831 This is the HCC combination, not atezolizumab alone or a lung-cancer chemotherapy combination. Existing liver disease and bleeding risk need review alongside possible immune side effects. Your team assesses each medicine separately.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Atezolizumab intravenous (IV)/subcutaneous (SC) + bevacizumab IV

Active medicines: atezolizumab + bevacizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis; individualise rescue treatment under NCIS minimal-risk/local policy.
- No routine premedication specified; assess prior administration reaction.
- Carry immunotherapy alert card and oncology contact plan.
- Manage varices, blood pressure (BP), urine-protein findings and procedure timing.
- Do not mask new significant diarrhoea with self-treatment; assess immune colitis and other causes.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

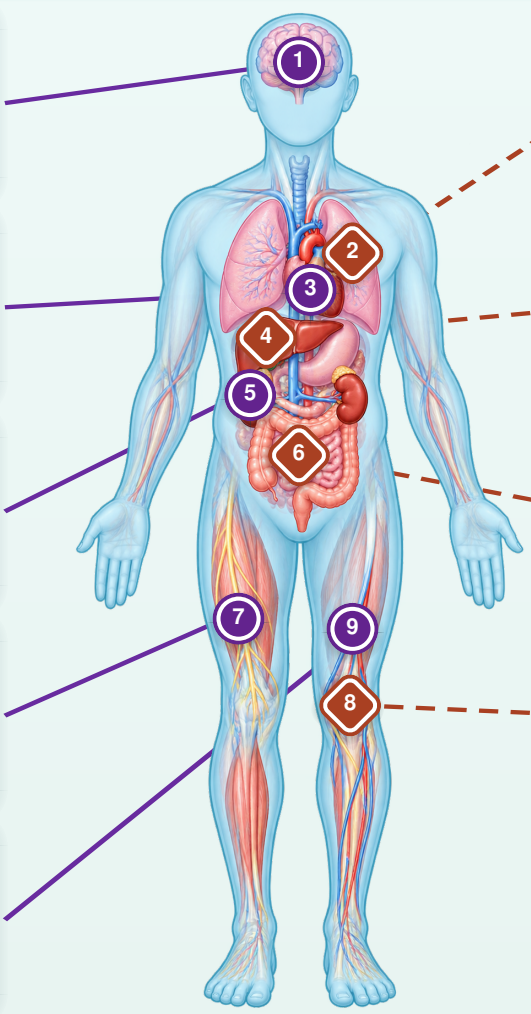
Atezolizumab + bevacizumab for hepatocellular carcinoma (HCC)

Atezolizumab is given first on Day 1 as an intravenous (IV) infusion or a subcutaneous injection into the thigh, followed by bevacizumab through a vein. Treatment is planned every 21 days while it is helping and side effects remain acceptable.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Hormones / brain

Atezolizumab

Tiredness, thirst, dizziness or a new headache may need hormone or neurological review.

2 Lungs

Atezolizumab

New cough or breathlessness needs prompt assessment for lung inflammation and other causes.

3 Heart

Atezolizumab

Chest pain, palpitations or breathlessness may reflect heart or surrounding-tissue inflammation.

4 Liver

Atezolizumab / underlying HCC

Yellow eyes, dark urine or blood-test changes need cause assessment; liver disease is not automatically immune hepatitis.

5 Kidneys / urine

Both medicines

Reduced urine or swelling needs review; immune kidney inflammation and bevacizumab protein loss require different assessment.

6 Bowel

Both medicines

Diarrhoea, blood or pain can indicate immune colitis; sudden severe pain can signal bowel perforation and needs emergency assessment.

7 Skin / nerves / muscles

Atezolizumab

Rash, blisters, weakness, numbness or muscle pain can be immune effects.

8 Joints

Atezolizumab

Painful swollen joints may need assessment for inflammatory arthritis.

9 Blood vessels / healing

Bevacizumab

Bleeding, clots, raised blood pressure or delayed wound healing need prompt review.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Accepted HCC organ positions are preserved. The medicines can cause overlapping symptoms; the team establishes the cause.

Call your oncology team immediately for new breathlessness, persistent diarrhoea, blood or black stools, yellow eyes, dark urine, severe tiredness or dizziness, thirst and frequent urination, unusual weakness, reduced urine or a spreading rash. Tell every healthcare professional that you receive immunotherapy, even after it stops. Vomiting blood, heavy bleeding, sudden severe abdominal or chest pain, stroke symptoms, seizure, collapse or major breathing difficulty need 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Recording pages are reusable and do not prescribe a number of cycles or authorise a restart.

Atezolizumab · PD-L1 immune checkpoint inhibitor

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Breathing / cough	□ Usual breathing	□ New cough or breathlessness: call promptly	□ Symptoms with usual activity: urgent review	□ Breathless at rest: urgent assessment	□ Severe breathing difficulty / collapse: 112/999
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Skin / nail changes	□ None	□ Mild change: report	□ Pain or daily tasks affected: call	□ Spreading infection or blistering: urgent review	□ Severe illness / collapse: emergency help
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Headache	□ None	□ Mild and short-lived: record	□ New / persistent headache: call	□ Severe headache / visual change: urgent help	□ Sudden worst headache / weakness: 112/999
Muscle / joint pain	□ None	□ Mild aches; normal activity	□ Pain limits tasks / sleep: call	□ Pain limits self-care: urgent review	□ Sudden severe deterioration: emergency help

Bevacizumab · Anti-VEGF antibody

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Bleeding / bruising	□ None	□ New small bruise / brief bleed: report	□ Repeated bleeding: call today	□ Bleeding that will not stop: urgent help	□ Heavy bleeding / collapse: 112/999
Headache	□ None	□ Mild and short-lived: record	□ New / persistent headache: call	□ Severe headache / visual change: urgent help	□ Sudden worst headache / weakness: 112/999
Urinary symptoms	□ Usual pattern	□ Mild change: report	□ Pain or frequent urination: call	□ Cannot pass urine / marked bleeding: urgent review	□ Collapse or severe illness: 112/999
Muscle / joint pain	□ None	□ Mild aches; normal activity	□ Pain limits tasks / sleep: call	□ Pain limits self-care: urgent review	□ Sudden severe deterioration: emergency help
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

Atezolizumab + bevacizumab for HCC

NCCP 00831 v2a · 21-day cycle

Atezolizumab is given first on Day 1 as an IV infusion or a subcutaneous injection into the thigh, followed by bevacizumab through a vein. Treatment is planned every 21 days while it is helping and side effects remain acceptable.

1 2 3 4 5 6

Your contact plan

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
Emergency: 112 / 999.

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00831/ · no patient information.

Draft for review · SACTCheck v0.84.24

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00831 v2a · official regimen**
   https://healthservice.hse.ie/documents/6432/831_v2a_Atezolizumab_Bevacizumab_Therapy.pdf
2. **EMA Tecentriq product information (intravenous (IV) and subcutaneous (SC))**
   https://www.ema.europa.eu/en/documents/product-information/tecentriq-epar-product-information_en.pdf
3. **EMA Avastin product information**
   https://www.ema.europa.eu/en/documents/product-information/avastin-epar-product-information_en.pdf
4. **IMbrave150 updated analysis: Cheng et al., 2022**
   https://pubmed.ncbi.nlm.nih.gov/34902530/
5. **NCIS Medical Oncology antiemetics V8: minimal-risk row**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
6. **HSE treatment side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
7. **NCI: adult primary liver cancer — alpha-fetoprotein (AFP) interpretation**
   https://www.cancer.gov/types/liver/hp/adult-liver-treatment-pdq